

Differential Diagnoses

Localized scleroderma (LS), scleromyxedema, nephrogenic systemic fibrosis, scleredema, and systemic sclerosis (SSc) overlap syndromes should be considered in the differential diagnosis. To differentiate primary from secondary Raynaud's phenomenon, nail fold capillaroscopy and autoantibody analysis are essential. Secondary causes such as vasculitis, thromboangiitis obliterans, or arteriosclerotic vascular disease should be excluded—evaluation may include tests such as the Allen test or fist closure test. Calcinosis cutis lesions must be distinguished from superficial ulcerations, although they may predispose to digital ulcers. Muscle biopsy can help determine the histological and ultrastructural features of myositis when present.

Treatment

Tailoring treatment to the individual patient requires accurate assessment of disease subset, organ involvement, and disease activity. In recent years, an organ-based approach has led to significant pharmacologic advancements, substantially improving prognosis and quality of life for specific patient subgroups.